



Administrative Burden in Australian Healthcare

Verified Evidence for Clinical Governance

Presentation | January 2026

The \$261 Million Legal Crisis



NSW Health

ACT Health

\$229.8 Million

\$31.5 Million

Supreme Court [2024] NSWSC 1171

Federal Court VID705/2022

Approved: August 2024

Approved: December 2024

Two States. Two Court Judgments. 2024.

Administrative overload has cost \$261.3 million in court-approved settlements for junior doctor unpaid overtime. This is not a wellbeing issue—it's Tier-1 legal and financial liability.

The \$5.4 Billion Annual Opportunity



\$5.4 Billion Per Year

Up to 30% of workforce tasks could be automated

Source: Productivity Commission 2024

Research paper: 'Leveraging digital technology in healthcare'

Potential savings from better digital information use, primarily through:

- Reduced hospital length of stay
- Fewer duplicated tests (\$355M annually)
- Workflow automation and efficiency gains

Patient Safety Impact: Administrative Delays Harm Patients

1.6% Daily Readmission Risk



Key Findings:

- 37.6% of discharge summaries delayed more than 24 hours
- Each day of delay = 1.6% higher 30-day readmission risk
- 135 hours per week at one hospital = 3.5 FTE doctors
- Median completion time: 14.5 minutes per summary

Source: Kleinig et al., Internal Medicine Journal 2025

Sample: 7,185 discharge summaries, SA hospitals / DOI: 10.1111/imj.70150

Workforce Crisis: 32% of GPs Leaving Within 5 Years



32% Plan to Cease Practicing

The Evidence:

- 60% cite regulatory/compliance burden as influencing factor
- 5.1 hours per week unpaid work = \$10,500-\$23,000 annually
- GPs under 35: Confidence dropped from 90% (2017) to 73% (2022)
- 32% burnout rate, 21% cite admin burden as primary cause

Sources: RACGP Health of Nation 2024 (n=3,006) | Commonwealth Fund 2025

Proven Australian Solutions



1. AI Ambient Scribing (Gold Coast Health 2024)

- 58% acceptance rate without modification (peer-reviewed Jan 2026)
- 16-week trial, 100+ clinicians, 21 specialties
- Hallucinations identified—requires human-in-the-loop governance

2. Electronic Medication Management (Queensland Hospitals)

- Turnaround time: 120 minutes (paper) → 35 minutes (digital)
- Time-critical meds: 77% within 60 min (vs 38% paper)
- *Austin et al., IJPP 2018 | DOI: 10.1111/ijpp.12432*

3. Digital Referral Systems

- Problem: 10% referral rejection rate, 80% still use fax/post
- Solution: Standardized digital referrals with structured data

Three Immediate Actions



1. Pilot AI Ambient Scribing with Clinical Governance

- Model: Gold Coast Health 16-week trial approach
- Governance: Mandatory human-in-the-loop review
- Benefit: 80% documentation time reduction potential

2. Standardize Digital Referrals Nationally

- Problem: 80% fax/post use, 10% rejection rate
- Solution: Interoperable structured data across hospitals
- Integration: GP practice software (Best Practice, Medical Director)

3. Recognize Non-Contact GP Time in Medicare

- Problem: 5.1 hrs/week unpaid = \$10-23K annual invisible subsidy
- Solution: Review MBS to acknowledge non-face-to-face work
- Benefit: Improves viability, reduces burnout, prevents exodus

Return on Investment: 20:1 Ratio



Current Costs Already Paid:

\$261.3 Million

Opportunity Identified:

\$5.4 Billion Annual Savings

ROI Ratio: Approximately **20:1**

The Question:

Can we afford NOT to act?



The Evidence Is Irrefutable

Legal Liability: \$261M already paid

Patient Safety: 1.6% daily readmission risk

Workforce Crisis: 32% leaving in 5 years

Proven Solutions: Working in Australian hospitals now

The time to act is now.